

HAVEN

[Housing](#)[Assistance](#)[Vital](#)[Emergency](#)[Network](#)**HOUSING****TRANSPORT****MEDICAL**

Disaster Response TPA

Complete continuity of care coordination for health plan members during catastrophic events



DEE DAVIS INC.

Contract Management Firm | Healthcare TPA Services

[EDWOSB](#)[WOSB](#)[WBENC](#)[MBE](#)[SBE](#)

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SECTION 01

The Crisis No One Is Ready For

When disaster strikes, health plan members face a cascade of failures across housing, transportation, and medical services.

2.5M+

Members displaced by
Hurricane Ian (2022)

\$113B

Insured losses from 2023
US disasters

30-90

Days of displacement per
major event

Three Systems Fail Simultaneously



Housing Collapses

Members scatter to
shelters, hotels,
relatives — no stable
address for care
delivery

- Home destroyed or uninhabitable
- Evacuated 100+ miles away
- No stable address on file
- Care coordination impossible



Transport Disappears

Members can't reach
appointments — their
car is gone, NEMT
providers are
overwhelmed

- Personal vehicle damaged/lost
- NEMT providers shut down
- Unfamiliar area, no options
- Medical appointments missed



Medical Continuity Breaks

Critical services
interrupted — meds,
dialysis, home health,
DME all disrupted

- Prescriptions lost/expired
- Dialysis center flooded
- Home health aide can't reach
- DME destroyed or left behind

The Regulatory Reality

MCOs remain responsible for member continuity regardless of disaster. CMS Section 1135 waivers provide flexibility, but **the obligation to provide care doesn't disappear**. Every missed dialysis treatment, every empty prescription, every unreachable member is a compliance risk.

SECTION 02

The HAVEN Solution

One TPA relationship that coordinates all three pillars of disaster continuity.



How It Works

Phase	Housing	Transportation	Medical Continuity
Pre-Disaster	Hotel/rental network contracted, rates locked, capacity committed	Rideshare healthcare networks integrated, evacuation routes mapped	Home health agencies, DME suppliers, pharmacies networked
Activation	Confirm hotel availability, prep for member placements	Confirm driver capacity, pre-stage evacuation transport	Alert agencies, confirm capacity at alternate locations

Member Intake	Where is member staying? Need housing?	Do they have transport? Medical appointments pending?	What services were they receiving? Critical needs?
Service Delivery	Book hotel/rental, track duration, manage extensions	Schedule rides to appointments, dialysis, pharmacy	Assign home health, deliver DME, coordinate Rx fills
Recovery	Transition to permanent housing or return home	Continue transport until member vehicle restored	Transition back to original providers when able

SECTION 03

Housing Services

The "H" in HAVEN — the foundation everything else builds on.



Temporary Housing Coordination

Stable shelter enables stable care

Without a stable address, care coordination collapses. Home health can't find the patient. DME can't be delivered. Prescriptions have nowhere to go. **Housing is the foundation that makes the other two pillars possible.**

WHAT DDI COORDINATES

- Hotel placement (individual and family rooms)
- Extended stay facilities for longer displacement
- Short-term rental coordination (vacation rental platforms, property managers)
- ADA-accessible housing for mobility-impaired
- Pet-friendly options (members won't leave pets)
- Duration tracking and extension management

NETWORK PARTNERS

- Major hotel chains (national and regional brands)
- Extended stay brands (Extended Stay America, Homewood)
- Regional property managers
- Corporate housing providers
- FEMA TSA coordination (where applicable)

Housing Volume Projections



Who Pays

Payer	Coverage	Limits
Homeowners/Renters Insurance	Additional Living Expenses (ALE)	Typically 20-30% of dwelling coverage
FEMA TSA	Transitional Sheltering Assistance	30-180 days, room + tax only
MCO (for medical necessity)	Housing to enable care delivery	Case-by-case authorization
State Emergency Programs	Varies by state	Varies

SECTION 04

Transportation Services

The mobility layer that connects members to care.



Medical & Evacuation Transport

Getting members where they need to be

Displaced members can't drive to dialysis 60 miles away. Their car may be destroyed. Local NEMT providers may be shut down. **Transport is the bridge between where members are and where care is.**

WHAT DDI COORDINATES

- Pre-disaster evacuation (vulnerable populations)
- Medical appointment transport
- Dialysis transport (3x weekly, critical)
- Pharmacy pickup/delivery coordination
- Wheelchair and stretcher transport
- Return transport when areas reopen

NETWORK PARTNERS

- Rideshare healthcare platforms (API integration)
- NEMT network partners (API integration)
- Regional NEMT providers
- Wheelchair van services
- Medical transport companies
- Charter bus (mass evacuation)

Transportation Volume Projections

2,000-
10,000

Transport trips per major event

30-90

Days of elevated transport demand

\$25-75

Average trip cost

Who Pays

Payer	Coverage	Notes
Medicaid (NEMT benefit)	Non-emergency medical transport	Standard NEMT billing codes
Medicare Advantage	Supplemental transport benefit	Many MA plans include
Commercial Health Plans	Varies by plan	Often covered for dialysis, chemo
FEMA/State Emergency	Evacuation transport	Pre-disaster evacuation programs

SECTION 05

Medical Continuity Services

The critical care coordination that keeps members alive and healthy.



Rx, DME & Home Health Coordination

Continuity of care wherever members land

Dialysis patients can't miss treatments. Oxygen-dependent members can't survive without their concentrator. Diabetics need insulin. **Medical continuity is the reason HAVEN exists — people die without it.**

WHAT DDI COORDINATES

- Prescription refills and emergency fills
- Prior authorization overrides (Section 1135)
- Pharmacy delivery to temporary locations
- DME replacement (oxygen, wheelchairs, CPAP)
- Home health aide reassignment
- Dialysis center coordination

NETWORK PARTNERS

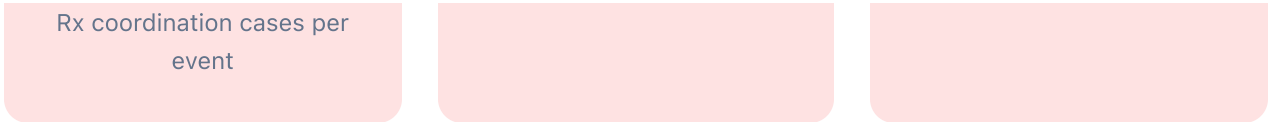
- Mail-order pharmacies
- Local pharmacy chains
- DME suppliers with emergency inventory
- Licensed home health agencies
- Dialysis center networks (DaVita, Fresenius)
- Specialty care providers

Medical Continuity Volume Projections

500-
2,000

100-500
DME replacements per
event

200-800
Home health
reassignments



Who Pays

Service	Payer	Billing Mechanism
Prescription medications	PBM / Health plan pharmacy benefit	Standard Rx billing (NDC codes)
DME (oxygen, wheelchair, CPAP)	Health plan DME benefit	HCPCS codes, rental or purchase
Home health services	Health plan home health benefit	HCPCS codes, hourly or visit-based
Dialysis	Health plan (often Medicare primary)	Standard dialysis billing
Coordination/case management	MCO (admin fee to DDI)	PMPM or per-case fee

SECTION 06

Scenario: All Three Pillars in Action

See how Housing, Transportation, and Medical Continuity work together for one member.

Maria, 67 — Dialysis Patient, Oxygen-Dependent

Maria lives in Houston. She receives dialysis 3x weekly and uses supplemental oxygen. Hurricane makes landfall. Her apartment floods. She evacuates to her daughter's house in Dallas, 240 miles away.



HOUSING

Maria is at daughter's house — stable address confirmed. HAVEN logs location, updates member record, enables care delivery.



TRANSPORT

Maria's car is flooded. HAVEN schedules medical transport for dialysis 3x weekly at Dallas center. Rides confirmed for next 30 days.



MEDICAL

HAVEN coordinates dialysis at DaVita Dallas. Portable oxygen concentrator delivered to daughter's address. Prescriptions transferred to Dallas pharmacy.

Cost Comparison

Outcome	Without HAVEN	With HAVEN
Dialysis continuity	Misses 2 treatments → ER visit (\$5,000+)	Uninterrupted dialysis at Dallas center
Oxygen supply	Without concentrator → hospitalization (\$15,000+)	Portable O2 delivered within 24 hours (\$200)
Transportation	Can't reach appointments → health deteriorates	30 rides scheduled (\$45/ride × 12 = \$540)

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13/20

Rx continuity	Runs out of meds → crisis	Transferred to Dallas pharmacy, filled same day
Total	\$20,000+ in avoidable costs	~\$800 in coordinated services

The Math

One member, one disaster, \$19,000+ in avoided costs. Multiply by 500 high-risk members per MCO per event. **HAVEN doesn't cost money — it saves money.**

SECTION 07

Partnership Opportunities

Whether you're an MCO, transport provider, housing partner, or medical services organization — there's a role for you in HAVEN.



Housing Partners

- Hotels and extended stay properties
- Property management companies
- Corporate housing providers
- Short-term rental networks

Your role: Provide rooms at pre-negotiated disaster rates. DDI handles member placement and billing.



Transport Partners

- Credentialed rideshare healthcare networks
- Regional NEMT providers
- Wheelchair transport companies
- Charter bus operators

Your role: Provide rides via API or dashboard. DDI handles scheduling, case management, MCO billing.



Medical Partners

- Home health agencies
- DME suppliers
- Pharmacy networks
- Dialysis center networks

Your role: Provide services to displaced members. DDI handles referrals, tracking, care coordination.

For MCOs / Health Plans

HAVEN provides a single TPA relationship that covers all three disaster continuity pillars. Instead of scrambling to coordinate housing, transport, and medical services during a crisis, you have one partner with pre-built networks ready to activate.

- Single contract for complete disaster continuity
- Pre-built networks — no disaster-time vendor negotiations
- Real-time member tracking and status reporting
- Compliance documentation for CMS/state audits
- Cost savings through coordinated care (avoided ER, hospitalizations)

→ EDWOSB partner for diversity procurement goals

DDI's Role Across All Partnerships

Function	DDI Responsibility
Network Development	Build and maintain partner networks in disaster-prone regions
Contract Management	Negotiate rates, manage agreements, ensure compliance
Member Intake	Receive displaced member lists, conduct needs assessments
Service Coordination	Match members to appropriate services across all three pillars
Dispatch & Tracking	Schedule services, monitor delivery, track completion
Quality Assurance	Monitor partner performance, resolve issues, ensure SLAs
Billing & Reporting	Bill MCOs, pay partners, provide compliance documentation

SECTION 08

About Dee Davis Inc.

HAVEN builds on DDI's proven healthcare TPA infrastructure.

Current DDI Operations



Drug Testing C/TPA
DOT and non-DOT
program administration



NEMT Coordination
Medical transportation TPA
services



Occupational Health
Physicals, fit testing,
screenings

Why DDI for Disaster Response

- **Proven TPA Model:** We already coordinate networks of credentialed providers for healthcare clients
- **Healthcare DNA:** We understand HIPAA, MCO contracts, Medicaid requirements, and healthcare billing
- **Network Management:** Our core competency is managing vendor networks — exactly what disaster response requires
- **Agility:** Small business speed. Decisions in hours, not weeks. No corporate bureaucracy.
- **EDWOSB Certification:** Woman-owned small business status supports MCO diversity goals

Company Information

Legal Name	Dee Davis Inc.
Headquarters	755 W. Big Beaver Rd., Suite 2020, Troy, MI 48084
CAGE Code	8UMX3
UEI	HJB4KNYJVGZ1

SAM.gov	Active
Certifications	EDWOSB, WOSB, WBENC, MBE, WBE, SBE

SECTION 09

Next Steps

Let's build the network that protects members when disaster strikes.

Ready to Join HAVEN?

Whether you're an MCO seeking disaster continuity or a service provider wanting to join our network, let's talk.



Housing
Partners



Transport
Partners



Medical
Partners



MCOs

For MCOs

- Discovery call to discuss current disaster preparedness and gaps
- Member population analysis (high-risk members, geographic distribution)
- Pilot program proposal for upcoming hurricane season
- Full deployment roadmap

For Service Partners

- Network enrollment discussion
- Rate and capacity negotiations
- Integration planning (API, dashboard, or concierge)
- Disaster activation protocols

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